

THE AESTHETICS PILOT / MEDLYFE REGENERATIVE AESTHETICS

Service, Pricing and Training

A medically-led menu, benchmark pricing, and the plan to train the team.

PREPARED FOR

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BY

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BASIS

Grounded in benchmark research; figures to firm on local pricing and rules

STATUS

Draft for discussion



01 / THE APPROACH

Injectables at the core, regenerative as the edge, clinicians only.

The menu anchors on injectables, botulinum toxin and dermal fillers, which are the primary revenue driver of any serious aesthetics practice, capturing more than half of med-spa revenue and carrying the best margins (50 to 70 percent gross). Around that sits a regenerative and wellness layer, PRP, bio-remodelling and IV infusions, that differentiates the brand and carries strong markup. Everything is delivered by clinicians, doctors and nurses, which suits a premium, medically-led positioning and is what keeps it safe.

Pricing here is benchmarked from premium markets (Dubai, South Africa, the US) converted to naira at about NGN 1,500 to the dollar. NGN is volatile and there is little public Lagos price data, so treat these as a defensible starting point to validate against the Medlyfe list, Dr Kpaduwa's view, and one or two local competitors.



02 / THE SERVICE PROFILE

What to launch, and what to differentiate on.

Injectables (the revenue anchor)

- Anti-wrinkle (botulinum toxin), dermal fillers, bio-remodelling (Profilo), skin boosters and mesotherapy, and PDO threads.

Regenerative and wellness (the differentiator)

- PRP for skin and hair, exosomes as a premium add-on, and IV wellness infusions. Low consumable cost, high perceived value, and a natural tie to the Medlyfe wellness brand.

Skin (the repeat-visit engine)

- Medical facials, chemical peels, microneedling and RF microneedling, and laser. These keep patients on a cycle between injectable appointments.

Launch with the injectables and the regenerative layer, they are the margin and the story. Add device-heavy lines (advanced laser platforms) only as demand proves out, since they tie up capital. Exosomes and threads sit with the doctor, not the nurse.



03 / THE PRICE LIST

Indicative Lagos pricing, benchmark-derived.

A premium list, positioned between South African and Dubai price points. The final column ties each treatment to who delivers it under the training model.

Treatment	Indicative (NGN)	Delivered by
Aesthetic consultation (redeemable)	50,000	Doctor or nurse
Anti-wrinkle, per area	180,000	Nurse or doctor
Anti-wrinkle, upper face (3 areas)	420,000	Nurse or doctor
Dermal filler, per syringe	450,000	Doctor; nurse (basic areas)
Bio-remodelling (Profillo), per session	450,000	Nurse or doctor
Skin boosters / mesotherapy	250,000	Nurse
PDO threads (non-surgical lift)	from 700,000	Doctor / Dr Kpaduwa
PRP, skin or hair (per session)	250,000	Nurse or doctor
Exosomes (premium add-on)	from 300,000	Doctor
IV wellness infusion	120,000	Nurse (doctor prescribes)
Medical facial	90,000	Nurse
Chemical peel	120,000	Nurse
Microneedling / RF microneedling	200,000	Nurse
Laser (per session)	150,000	Nurse



04 / MARGINS AND CONSUMABLES

Where the money is, honestly.

Injectables carry 50 to 70 percent gross margin at proper pricing (consumables 30 to 40 percent of the fee). The regenerative layer is even better on margin; bio-remodelling is the exception because the product itself is expensive.

Line	Indicative consumable cost	Gross margin
Anti-wrinkle (toxin)	Low per area	About 70% or better
Dermal filler	About 150,000 to 200,000 / syringe	About 55 to 65%
Bio-remodelling (Profillo)	About 286,000 / syringe (pricey)	About 40%, protect the price
PRP	About 33,000 / treatment	Very high, 80% or better
Skin, peels, facials	Low	High

These are gross margins before staff, power and overhead. Consumables are imported, so landed cost carries FX and lead time, order early and price with a buffer. The net margin in a diesel-powered Lekki clinic is lower than the gross and is what the pilot financials track.



05 / MEMBERSHIP (OPTIONAL FOR THE PILOT)

A light way to seed repeat behaviour.

Premium aesthetics elsewhere runs tiered memberships (roughly at 70 to 80 percent of retail, with a 10 to 25 percent member discount, protecting a 40 percent minimum margin). For the pilot, keep it to one simple tier to test appetite, not a full programme.

- **A founders' membership:** a modest annual fee for a member discount, priority booking, and a complimentary facial or skin review. Optional, and easy to switch off.
- The point during the pilot is to learn whether members return on a cycle, not to build the full three-tier structure yet.



06 / TRAINING AND CREDENTIALING

Two tiers, credentialed to a real standard.

This is how the service runs safely to Dr Kpaduwa's standard while she is away. She trains and credentials a two-tier clinical team, doctor and nurse, against a recognised competency milestone, and signs off each tier before independent delivery.

Tier	Delivers	Signed off to
Doctor (on-site clinical anchor)	Advanced filler, threads, cannula work, regenerative procedures, prescribing, and first-line complication management. The accountable clinician when Dr Kpaduwa is abroad.	Full credentialing + complication management
Nurse (core delivery)	Anti-wrinkle, basic filler, skin boosters, PRP, and the skin treatments (peels, microneedling, facials, laser), all under protocol.	The 10 + 10 milestone per injectable modality

The competency milestone (recognised standard)

- Per the JCCP / CPSA competency framework, each injector logs a minimum of **10 observed plus 10 supervised cases per modality** (toxin and filler counted separately) before independent practice. Dr Kpaduwa observes, supervises and signs off.
- Written protocols and a tiered scope define exactly what each tier does solo versus what waits for the doctor or for Dr Kpaduwa's visits.
- Delivered in the next two weeks while she is resident, front-loaded before her maternity, then maintained on tele-supervision and audited on her visits.



07 / SAFETY GOVERNANCE

Why the doctor is not optional.

The research is unambiguous on one point: a lead who is abroad cannot be the accountable prescriber. Hyaluronidase, the drug that reverses a filler emergency, is prescription-only, and remote prescribing for elective cosmetic work is not acceptable practice. So a resident doctor who can prescribe and manage complications is the safety backbone that makes the whole delegated model legal and safe. That is the real reason the doctor tier exists.

The backbone the clinic must have

- **An accountable on-call prescriber on the ground** (the resident doctor), reachable within a defined timescale. Tele-support from Dr Kpaduwa is for oversight and complex cases, not for prescribing or emergencies.
- **Hyaluronidase stocked on site at all times**, with a documented vascular-occlusion protocol: high-dose pulsed hyaluronidase (about 1,500 units over the affected artery), re-dosed every 15 to 20 minutes on capillary-refill reassessment.
- **An anaphylaxis kit and written emergency protocols**, with defined escalation and supervisor-contact timescales, drilled with the team.
- **Audit and review:** Dr Kpaduwa reviews cases remotely and on her visits; competency is re-checked, not assumed.



08 / NIGERIA: WHAT TO CONFIRM BEFORE LAUNCH

The honest gaps.

The benchmarks and standards above are drawn from Dubai, South Africa, the US and UK best practice; there is little reliable public Lagos-specific data, so a few things must be confirmed locally before the price list and the model are final. These are quick to close and worth closing properly.

- **Who may legally inject** in Nigeria, and the role and registration of the prescriber (MDCN, NMCN).
- **Import and product registration** (NAFDAC) for toxin, fillers, hyaluronidase and exosomes, this affects lead time and cost.
- **The prescriber question:** whether Dr Kpaduwa can be the accountable prescriber from abroad, or a locally-registered doctor must hold and authorise hyaluronidase, this likely confirms the resident-doctor requirement.
- **Local pricing and willingness to pay:** validate the list against one or two Lagos competitors and the Medlyfe base.



09 / NEXT STEPS

The two-week training and set-up sprint.

- Confirm the launch menu and the price list against the Medlyfe list and Dr Kpaduwa's view.
- Assign the resident doctor and the nurse, and start the observed-and-supervised training now, before the maternity break.
- Write the protocols and the emergency and vascular-occlusion pack; stock hyaluronidase and the anaphylaxis kit.
- Order the imported consumables early, and confirm the Nigerian regulatory points.

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Pricing is benchmark-derived (Dubai, South Africa, US) converted at about NGN 1,500 per USD (mid-2026); refresh before publishing. Competency and safety standards follow the JCCP / CPSA framework and the CMAC hyaluronidase guideline as best-practice proxies, to be aligned with Nigerian regulation. Draft for Dr Itunu Akinware. Prepared by Consult for Africa.